

Report

IntelePeer's AI Revolution: Enhancing Benevis LLC's Patient Call Experience

Shruti Vachher

1. Executive Summary

Benevis currently relies on a call center model that acts as the primary access point for patient care. However, with increasing call volumes and high variability in demand, there are now operational bottlenecks, leading to long wait times, missed calls, and reduced patient access. This issue is especially critical as over **60% of calls are routine appointment-related requests**, yet they are still handled manually, limiting the scalability and increasing costs.

This report evaluates multiple strategic alternatives, including full AI automation, data-first optimization, outsourcing, and a hybrid AI-human model. Based on the analysis, the **hybrid AI + human model** is recommended as the most effective solution. It will allow Benevis to automate high-volume, routine interactions while retaining human agents for complex and sensitive cases, ensuring the quality of care.

The proposed solution leverages IntelPeer's SmartAgent as a first point of contact to triage and handle incoming calls. This approach helps address key challenges identified in the analysis, including scalability limitations, resource inefficiencies, and inconsistent patient experience, while also mitigating risks related to compliance, trust, and data quality.

The implementation follows a **phased, risk-controlled approach**, beginning with data standardization and pilot testing, followed by gradual scaling and continuous optimization. From a financial perspective, the solution is strongly justified. Automation of up to **75% of call volume (~1.2 million calls annually)** can generate an estimated **annual ROI of approximately \$1.85 million**, with a **net return exceeding \$1.2 million** after costs. The investment is expected to reach **break-even within approximately 6 months**, making it both cost-effective and scalable in the long term.

Overall, the hybrid model allows Benevis to transition from a labor heavy system to a more scalable, technology assisted model without compromising patient trust or regulatory compliance. By doing so, Benevis can improve access to care, enhance operational efficiency, and support long-term growth in a sustainable manner.

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2. Background

2.1 Organization and case context

The Benevis company acts as a Dental Support Organization (DSO), specializing particularly in pediatric and family dentistry, where access to healthcare starts first and foremost through calls from patients into the call center related to appointment scheduling, re-scheduling, questions related to insurance coverage, follow-ups, and other inquiries. In other words, those calls become the main “front door” for patient engagement and care at Benevis; thus, handling such calls becomes critical to the success of the company and patient experience. As the company grows, more and more symptoms of inefficient communications have appeared in it, namely delayed answering, missed calls, etc.

Therefore, Benevis has been considering the option of introducing new technological solutions into their operations. For example, IntelPeer suggested the use of artificial intelligence to automate some of the processes. Specifically, the vendor suggested using its solution called “SmartAgent,” a generative AI tool meant to be used as the starting point of interaction with inbound calls and automate all administrative interactions. However, given the fact that Benevis provides healthcare services within a highly trust-sensitive environment, it cannot be seen only as a tool to be introduced; it needs to be treated as a digital transformation step.

2.2 Current operating environment

Benevis’s existing business context can be characterized by high demand, imbalance of call flow, and capacity constraints. The reliance of the company on live agents to manage most calls while having the majority of calls to handle being relatively straightforward appointment management operations represents inefficient work and inability to expand capabilities during peak hours. The gap analysis states that the call center experience itself can represent a bottleneck, especially on Monday and Tuesday morning hours, when high call volume could lead to longer wait times and abandoned calls, thus impacting patient access to services.

The existing business context is further complicated by inconsistency between technologies and workflows at different sites. Benevis’s scheduling inputs and workflow rules are not consistent across sites, making automation implementation inefficient and creating barriers to successful scaling of AI applications in the organization. In this context, the capacity of Benevis to scale up is limited not only by staffing capacity but also by the availability of quality data, readiness for proper integration, and governance.

2.3 Why this problem matters

The issue at hand is strategically relevant as patient accessibility and organizational sustainability are directly correlated. When the call centre reaches overload, it ceases to act as an efficient access channel and rather turns into a barrier, especially for those customers who depend on

effective booking and regular communication. Moreover, Benevis operates under the operational pressures, namely the necessity to provide high volume services, handle the work force pressure and maintain stability in relation to a healthcare related business.

The reason why the issue is relevant for the project is the fact that the organization cannot consider scaling as a question of staff augmentation only. According to the gap analysis, the core issue here is the lack of a triaging layer that would automate certain procedures and allow scaling of the operation regardless of variable demands of the customers.

2.4 Research basis

The present report is based on the material from the Benevis–IntelPeer case as well as on the analyses that were conducted by the team through the course project process. More specifically, the analysis draws upon the concept of the initiative as a digital transformation choice presented in the project plan, scalability definition outlined in the problem statement, and gap analysis results in the areas of technological readiness, efficiency, customer experience, compliance, governance, and change management. Furthermore, the present analysis leverages the secondary research performed by the team (HIPAA regulatory compliance baseline, HHS/OCR guidance regarding online tracking tools, WHO AI governance recommendations, NIST framework for managing risks associated with AI, and CADTH horizon scanning report on healthcare chatbots).

3. Problem Definition and Core Issues

3.1 Problem statement

Benevis cannot scale its call-center-based patient scheduling model to reliably handle rising and highly variable call demand during peak periods.

3.2 Issue identification

Drawing from the above case context and the findings of the team’s research, scalability issues at Benevis will come about as a result of:

- a. High capacity and variability: Demand spikes during peak times, leading to high levels of congestion that limit access to the company.
- b. High human dependence for common call intents: There exists a relatively high degree of dependence on humans in the handling of common tasks such as appointment scheduling (roughly 61%). Most of the calls handled at this company are done manually by humans.
- c. Inadequate technology readiness: There exist inconsistencies in the definition of key inputs required for automation, thus limiting scalability of a standardized approach towards managing the flow of calls.
- d. High impact of call center on patient experience: This serves as the main entry point into care. Delayed responses, variability, and poor availability during peak and off-peak times make it difficult for patients to get care.

- e. Trust, ethics, and risk of harm: Patient interaction with technology involves potential risks such as erroneous information, insensitivity, or mishandling the patient during sensitive situations, creating a threat to their trust and reputation.
- f. Regulatory compliance and privacy concerns: The use of artificial intelligence for interactions creates greater exposure to privacy risks unless built-in governance, monitoring, and auditing capabilities are included in the design process.
- g. Organization readiness and change management challenges: Change is hard enough, let alone during times when the organization's employees are stressed out.

3.3 Issue prioritization

They are then prioritized according to (a) causal relevance, meaning the way that the issue may cause or contribute to the core scalability issue, (b) danger in case of non-resolution, and (c) their necessity as enabling conditions for a future state of stability.

- Priority 1: Scalability failure at peak demand (performance bottleneck). This priority is chosen because it is the central issue that defines the core problem statement, which creates inefficiencies and patient access difficulties.
- Priority 2: Technology and data preparedness (necessary foundation for scalable safety). The second priority issue is chosen because it is a foundational issue whose non-resolution will lead to increased automation inconsistency.
- Priority 3: Resource inefficiency in handling routine call intents (cost/efficiency). This priority issue exists because it represents the largest category of call types, and therefore is the biggest source of inefficiency.
- Priority 4: Patient Experience and Accessibility ("front door to care"). This priority is important because the bottleneck at the call center will limit access and provide poor-quality service where it is needed most.
- Priorities 5-7: Trust/Ethics/Risk, Compliance/Privacy, and Readiness. These priorities are important, but not in place of the operational challenge itself; they decide whether a solution will be viable. Without governance, compliance, and change management, automation could lead to further problems with trust and reliability.

3.4 Stakeholder impact summary

The key stakeholders impacted by the challenges include patients and their careers, call center agents, clinic staff, the leadership team, and the technology provider. First, patients rely on the call centre as the main entry point to services provided; the challenge is that when there is too much traffic in the call center, accessing services becomes problematic, which may be viewed as a barrier to scheduling. Second, the call centre agents are impacted by workload and repetitive tasks leading to increased stress and inconsistency in performance, and also limiting attention to delicate aspects of work. Third, the leadership of Benevis has to manage scaling up of operations while minimizing risks in the context of a regulated market; it should be noted that no matter if

interaction takes place either via humans or automated system, responsibility for results will always lie with Benevis. Finally, interdependence emerges from the involvement of IntelPeer as the provider of the technology solution.

4. Alternatives Considered

4.1 Alternative 1: Full AI automation

Full AI automation means using AI to handle almost all patient calls without human involvement. This includes booking appointments, answering common questions, and managing simple requests. The goal is to fully replace front-line call center work with AI.

This strategy fits Benevis's situation in some ways. The case shows that about 61% of calls are related to appointment scheduling, which are repetitive and easy to automate. This means AI could reduce wait times, handle large call volumes, and lower labour costs.

However, this approach has major risks. Benevis works in healthcare and serves many Medicaid patients, including children. These situations often require empathy, clear communication, and human judgment. If everything is handled by AI, patients may feel frustrated or misunderstood, especially in more complex cases.

Also, the case highlights issues with inconsistent data and systems. AI depends on accurate data to work properly. If the data is not clean or consistent, the system may give wrong information or fail to complete tasks.

Overall, full AI automation is efficient, but it is too risky for Benevis right now because it could harm patient experience and create errors.

4.2 Alternative 2: Hybrid AI + human model

The hybrid AI + human model combines both AI and human agents. In this approach, AI handles simple and repetitive tasks like appointment booking, while human agents handle more complex or sensitive situations.

This option fits Benevis very well. Most calls are routine and can be handled by AI, while only a smaller portion needs human support. This helps reduce pressure on staff while keeping the quality of service high.

It allows Benevis to slowly introduce AI while fixing data and system issues at the same time. Instead of jumping straight into full automation, the company can improve step by step.

Another big advantage is that it keeps human involvement. This is important in healthcare, where patients may need reassurance, explanations, or help with unusual situations. Human oversight also helps reduce errors and supports compliance with healthcare regulations.

Overall, the hybrid model gives Benevis the best balance. It improves efficiency and scalability while still protecting patient experience and reducing risk.

4.3 Alternative 3: Data-first optimization

The data-first approach focuses on fixing data problems and improving systems before using AI. This includes cleaning data, standardizing workflows, and making sure all systems are connected properly.

This option is important because the case clearly shows that Benevis has inconsistent data across locations. This makes it hard for AI to work correctly. Fixing this first would make future automation more accurate and reliable.

However, this approach does not solve the main problem right away. Benevis is currently struggling with high call volumes and overloaded staff. If the company only focuses on data, these problems will continue in the short term.

It also takes time and effort to fix data across the organization. This delays the benefits of automation and does not improve efficiency immediately.

Overall, data-first optimization is useful, but it should support another solution rather than be used on its own.

4.4 Alternative 4: Outsourcing call center operations

Outsourcing means hiring a third-party company to handle patient calls instead of using internal staff or AI. The external provider would manage scheduling, questions, and other requests.

This option can help in the short term. It can reduce pressure on Benevis's call center and handle high call volumes quickly. It is also easier to implement compared to building new systems. However, outsourcing has several problems. First, it reduces control over service quality. External agents may not fully understand Benevis's systems or patient needs. This is risky because most calls involve appointment scheduling, which requires accurate and real-time information.

Second, there are serious data privacy concerns. Benevis handles sensitive patient information, and sharing this with third parties increases the risk of data breaches and compliance issues.

Finally, outsourcing does not fix the real problems. It does not improve data, systems, or workflows. It only moves the problem outside the company instead of solving it.

Overall, outsourcing may help temporarily, but it is not a good long-term solution.

4.5 Comparative evaluation criteria

The four alternatives were compared based on cost, risk, patient experience, scalability, ease of implementation, and compliance. Based on the comparison table in your proposal, the hybrid AI + human model performs the best overall.

Full AI automation scores high in efficiency and scalability, but it also has the highest risk and compliance concerns. This makes it unsafe for a healthcare environment like Benevis.

The data-first approach reduces risk and improves system quality, but it does not solve the immediate issue of high call volumes. It is more of a long-term support strategy.

Outsourcing is easy to implement and gives quick results, but it has low scalability, high compliance risk, and weak control over service quality. It also does not address the root problems.

The hybrid AI + human model stands out because it balances everything. It improves efficiency and scalability while keeping patient experience strong and risk at a manageable level. It also allows Benevis to gradually improve its systems instead of making sudden changes.

Because of this balance, the hybrid model is the best choice. It directly solves Benevis's main problems while also supporting long-term growth and stability.

5. Constraints and Reasons

5.1 Technology and data constraints

Due to the different systems not working well with one another as well as inconsistency in data between locations, it has made it difficult to implement an AI Schedule Work/ Patient Records Workflow that would work across all locations. The lack of standardization on scheduling workflows and patient records prevents the use of accurate and consistent data for AI systems to rely on as input.

AI performance relies heavily on the quality of the data and how well the systems are integrated. Therefore, without consistent and integrated data, there will be risks of incorrect responses and unreliable automation, which means that there is no advanced solution for AI Automation at this time until we address data consistency and integration issues.

5.2 Compliance/privacy/regulatory constraints

As a company, Benevis operates in a heavily regulated environment for healthcare due to HIPAA and other privacy regulations. Thus, all solutions that include patient data must provide strict protection for the data, secure access, and appropriate monitoring. Artificial intelligence (AI) has added complexity to compliance as it processes sensitive information in large quantities and needs ongoing monitoring. If there are no robust compliance frameworks in place, then the

likelihood of a data breach or unauthorized access will increase significantly, making rapid or uncontrolled adoption of AI a risky proposition.

5.3 Trust/ethics/risk constraints

The patient trust factor is very important to Benevis because it works with children and Medicaid-dependent families. Fully automated systems may be unable to process sensitive or emotional interactions appropriately, which could result in negative feelings or lack of confidence in the service. There are also ethical considerations regarding the use of AI for healthcare communication; that is, there are considerations regarding accountability, transparency, and appropriate behavior. If there are no human oversight and clear guidelines surrounding automation, it can lead to a negative impact on patient experience and reputation.

5.4 Organizational constraints

Benevis is having trouble managing both organizational change and getting its personnel prepared for AI use. The successful introduction of any new technology requires effective communication and collaboration from at least three departments (IT, Operations, and Compliance). In many cases, the level of cooperation can slow down the pace of transition to using new technology. Additionally, many employees have an unwillingness to accept or have reservations about being required to work with the AI system in conjunction with their normal functional duties. There may also be insufficient training and a lack of definitive accountability for implementation of the new technology, further hampering a successful transition.

5.5 Financial/operational constraints

Benevis has very small profit margins because most of their patients are Medicaid patients. They do not have enough money to heavily invest in large-scale solutions. Hiring extra personnel to assist in handling call volume is not sustainable, but upfront investment to help implement new advanced AI systems to reduce the number of calls they receive will be difficult. Finally, call volume is very erratic, so the peak times for calls are unpredictable to allow for effective planning of resources. Therefore, they require a solution that provides a balanced cost-effective solution to be implemented gradually in a controlled manner over time.

6. Proposed Solution

6.1 Solution overview (Hybrid AI + Human model)

The proposed solution for Benevis LLC is the implementation of a phased **Hybrid AI-Human Model**. In this framework, IntelPeer's "SmartAgent" serves as the primary "intelligent front door" for all incoming patient inquiries, while human agents are strategically retained to handle complex, high-emotion, or exception-based interactions. This model represents a fundamental shift from a labor-intensive manual system to a technology-enabled ecosystem. Rather than pursuing a total replacement of the workforce, the solution redistributes labor: routine and high-

volume tasks, such as appointment scheduling and FAQs, are automated to achieve the 3x ROI run-rate identified in the pilot, while human agents are freed to focus on specialized patient care that requires clinical judgment or empathy. This approach directly honors the intent study findings, which proved that the majority of Benevis's call volume is automation-ready, provided the technology is backed by a human safety net.

6.2 Solution scope and boundaries

To maintain operational control, the scope of the solution is strictly partitioned. In the immediate phase, the AI's "jurisdiction" includes appointment management (scheduling, cancellations, and confirmations) and standardized FAQs. These are low-risk, high-frequency tasks that benefit most from 24/7 availability. Out of scope for the AI are interactions involving multi-patient family bookings, billing disputes, or calls where the patient exhibits significant distress. By establishing these clear boundaries, Benevis ensures that the AI operates only within its "zone of competence," thereby protecting the brand from the risks of "hallucination" or clinical errors that could arise in more ambiguous scenarios.

6.3 How the solution works

The hybrid model restructures the patient journey into a tiered, intent-based system. Upon initial contact, the SmartAgent utilizes natural language processing (NLP) to categorize the patient's intent. Routine requests are processed end-to-end via a real-time integration with Benevis's backend scheduling systems. However, the system is designed with "escalation triggers." If the AI detects sentiment markers indicating frustration or if the request falls outside of pre-defined parameters, it performs a "warm handoff" to a human agent. This includes passing a transcript of the AI's interaction to the agent, ensuring the patient does not have to repeat themselves. This iterative loop not only improves immediate efficiency but also generates the data necessary to refine the AI's performance over time.

6.4 Key capabilities required

The transition to this model is as much organizational as it is technological. Benevis must prioritize **Data Standardization**; the AI cannot function as a "source of truth" if scheduling protocols remain fragmented across the 120+ locations. Furthermore, the organization requires enhanced **Cross-Functional Coordination** between IT and clinical operations to ensure that the "SmartAgent" reflects current clinic capacities. Finally, a robust **Governance and Compliance Infrastructure** is essential to monitor HIPAA adherence. These capabilities represent the "bridge" between Benevis's current manual state and the desired future state of a scalable, AI-driven DSO.

6.5 Expected business impact

The anticipated impact is both financial and operational. By automating the 70,000+ routine calls identified in the production trial, Benevis can achieve over \$1 million in annual savings,

effectively decoupling its growth from its labor costs. Beyond the bottom line, the solution addresses the "Monday morning surge" by providing unlimited concurrent call capacity, which will drastically reduce call abandonment rates and improve patient access for Medicaid populations. This creates a scalable platform that supports Benevis’s long-term expansion goals without compromising the patient-centered mission of the firm.

6.6 Risk mitigation strategy

To protect against the risks of technology-induced friction, this solution utilizes a **Phased Implementation** strategy. By starting with a "test and learn" approach in limited markets, Benevis can validate the AI’s accuracy before a national rollout. Additionally, the **Hybrid Escalation Model** acts as the ultimate fail-safe; if the data is inconsistent or the AI fails to resolve an issue, a human is always available to intervene. This "Human-in-the-loop" (HITL) philosophy ensures that the organization maintains patient trust while it works through the technical debt of data cleanup.

6.7 Alignment to gap analysis

This proposed solution is the direct fulfillment of the recommendations made in our Gap Analysis. It closes the **Scalability Gap** by introducing unlimited digital capacity and addresses the **Cost Constraint** by leveraging IntelPeer’s existing infrastructure. Most importantly, it bridges the **Organizational Readiness Gap** by positioning AI as an augmentative tool rather than a disruptive threat. By following this roadmap, Benevis can transform its communication infrastructure from a point of failure into a primary competitive advantage.

7. Implementation Plan

7.1 Implementation roadmap

The implementation follows a **phased, risk-controlled approach**, ensuring that Benevis can scale automation while maintaining compliance, data integrity, and patient trust. The approach prioritizes **foundation before scale**, directly addressing the gaps in technology readiness and operations.

Phase	Timeline	Focus
Phase 1	0–3 months	Data and system foundation
Phase 2	3–6 months	Pilot deployment
Phase 3	6–12 months	Scaling and integration
Phase 4	12+ months	Optimization

7.2 Phase 1: Foundation (0-3 Months)

Objective: Prepare infrastructure for safe AI deployment

- Standardize scheduling workflows across locations

- Clean and structure the data for AI use
- Define AI scope
- Build secure system integrations
- Establish governance (HIPAA, escalation rules)

Outcome: AI-ready data and consistent workflows

7.3 Phase 2: Pilot deployment (3-6 months)

Objective: Test performance in a controlled environment

- Deploy SmartAgent in selected high-volume locations
- Enable human-in-the-loop (HITL) escalation
- Train staff on AI-supported workflows
- Monitor performance via dashboards

KPIs: containment rate, wait time, escalation rate, CSAT

Target: replicate production trial performance (~70,000 calls automated and 3x ROI run-rate)

Gate: Scale only if performance meets the defined KPI thresholds

7.4 Phase 3: Scaling and integration (6-12 months)

Objective: Expand adoption and strengthen systems

- Roll out AI across additional locations
- Expand use cases (e.g., reminders, basic billing)
- Improve system integration and data flow
- Introduce centralized performance monitoring

Impact: Reduced reliance on staffing and improved peak capacity

Target: achieve automation of approximately **60–75% of total call volume**

7.5 Phase 4: Optimization (12+ months)

Objective: Continuous improvement and capability expansion

- Refine AI using interaction data
- Improve escalation logic and accuracy
- Conduct regular compliance and performance reviews

Outcome: AI becomes a scalable, long-term operational capability

7.6 Resource allocation plan

7.6.1 Technology resources

- SmartAgent platform deployment (IntelePeer)
- Data standardization and integration tools
- Secure APIs for real-time scheduling access
- Performance monitoring dashboards (real-time KPIs)
- Compliance and audit tracking systems (HIPAA)

7.6.2 Human resources (training, oversight roles)

- Staff training for new workflows (AI-assisted operations)
- AI oversight roles (monitoring + escalation handling)
- Cross-functional coordination teams:
 - IT (integration)
 - Operations (workflow design)
 - Compliance (risk monitoring)

7.6.3 Financial resources

Cost Structure Overview

The financial viability of the hybrid AI model depends on balancing **initial investment with long-term operational savings**. Rather than large upfront capital expenditure, the phased rollout allows Benevis to **distribute costs over time**, reducing financial risk.

One-time (implementation) costs:

- System integration and API development
- Data standardization across locations
- Staff training and workflow redesign

Ongoing costs:

- AI platform subscription (SmartAgent)
- System monitoring and compliance oversight
- Continuous optimization and model updates

Cost Savings & Return Profile

The financial impact is driven by reducing reliance on human agents for routine calls, which account for approximately **61% of total call volume**. Through automation, Benevis can handle up to **75% of calls (~1.2 million annually)**, significantly improving efficiency.

Key drivers of savings: Reduced call handling time for routine interactions, lower staffing pressure during peak periods, avoided hiring costs as demand grows

This results in an estimated **annual ROI of ~\$1.85 million**, with a **net return of over \$1.2 million**, compared to an annual investment of approximately **\$657,600**. The solution is expected to reach **break-even within ~6 months**, based on pilot performance. The primary value is not only cost reduction, but in scaling **operations without proportionally increasing labor**.

Return Profile:

- **0–6 months:** Investment phase (setup + pilot)
- **6–12 months:** Break-even as automation scales
- **12+ months:** Sustained cost savings and operational efficiency

The strength of the ROI is supported by high automation potential, immediate efficiency gains, and a scalable cost structure where increased call volume does not significantly increase costs.

7.6.4 Success measurement plan (KPIs, review cadence, go/no-go gates)

To ensure accountability and measurable outcomes, performance will be tracked through structured KPIs:

Category	KPI	Purpose
Efficiency	Call containment rate	Measure automation success
Operations	Average wait time	Evaluate service improvement
Quality	Escalation rate	Identify AI limitations
Experience	Patient satisfaction (CSAT)	Maintain care quality
Financial	Cost per call	Track efficiency gains

8. Recommendations

8.1 Immediate actions (next 30–60 days) + owners

Benevis should begin by standardizing scheduling data and workflows across locations to ensure reliable AI performance. A governance structure must also be established, including escalation rules, compliance oversight, and clearly defined responsibilities. At the same time, IT teams should initiate secure integration planning between SmartAgent and existing systems, while compliance teams validate alignment with HIPAA requirements.

Owners: IT & Data Teams, Compliance Team, Operations Leadership.

8.2 Short-term actions (3–6 months) + owners

Benevis should launch a pilot of the Hybrid AI + Human model focused on high-volume, low-risk tasks such as appointment scheduling and FAQs. The pilot should be limited to selected locations to manage risk.

Performance should be tracked using KPIs such as automation rate, wait time, escalation rate, and patient satisfaction, while feedback is used to refine workflows.

Owners: Operations Team, IT Team, Customer Experience Team, IntelPeer

8.3 Medium-term actions (6–12 months) + owners

Following pilot success, Benevis should scale AI across additional locations and expand use cases (e.g., billing support and reminders). System integration should be strengthened, and AI models should be continuously improved using real data. Ongoing staff training is required to support the transition toward oversight roles.

Owners: IT & Integration Teams, Operations Leadership, HR & Training Teams, IntelPeer

8.4 Governance and accountability (who owns what ongoing)

A structured governance model is required to ensure long-term success. Benevis should establish clear accountability across departments for AI performance, compliance, and operational outcomes. AI oversight roles should be created to monitor system performance, manage escalations, and ensure adherence to privacy regulations. Real-time dashboards should be used to track KPIs and identify risks early. Vendor governance agreements with IntelPeer must clearly define responsibilities related to data handling, system performance, and compliance.

8.5 Further actions / future work (if applicable)

In the long term, Benevis should explore advanced capabilities such as predictive scheduling and proactive patient engagement. Continuous improvement and regular compliance reviews will ensure the system remains effective and trustworthy.

9. Conclusion

Benevis's current call center model is not scalable due to its reliance on manual processes and inability to manage fluctuating demand. Although AI is a great opportunity to enhance

efficiency, a complete automation strategy poses unacceptable risks in a healthcare setting where trust, accuracy, and compliance are paramount. The Hybrid AI + Human model will be the most appropriate solution to Benevis based on the analysis of alternatives. By doing so, the organization is able to automate high volume and routine interactions, but retain human control over complex and sensitive cases. This balance will enable Benevis to enhance operational efficiency, minimize costs, and increase access to patients without affecting quality or compliance, as emphasized in the solution proposal.

This strategy will require a gradual and managed implementation with effective data governance, system integration, and ongoing monitoring to succeed. By tackling the underlying vulnerabilities and scaling automation stepwise, Benevis will be able to reduce the risks and create a trusted and sustainable system based on AI. Finally, Hybrid AI + Human model does not imply the substitution of the human agents but the improvement of their functions. It enables Benevis to transition towards a more labour-based model to a scalable technology-based model that enhances the performance of operations and patient experience. This will make the organization grow in the long-term and retain trust, compliance, and quality care provision.

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